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TITLE: Pharmacovigilance Advisory - Reported Dizziness with CardioZen
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PHARMACOVIGILANCE ADVISORY

Advisory: SAFETY_DOC_009
Issued: 10 April 2026
Unit: Pharmacovigilance
Action: Awareness. No change to monitoring practice.

| SYNTHETIC DOCUMENT - NOT REAL MEDICAL GUIDANCE. Fabricated for a
| software demonstration. CardioZen is an invented medication. Nothing here
| describes a real product or should inform any clinical decision.

1. Summary of the signal

Between October 2025 and March 2026 the Unit received thirty-one reports of dizziness in patients receiving CardioZen. Reports originated from all three sites and from two community pharmacy partners. Onset was described as within the first fortnight of therapy in the majority of reports and as transient in twenty-four of the thirty-one.

2. Assessment

The Unit assessed the reports for a dose relationship and did not find one. Twelve of the thirty-one reports described concurrent initiation of a second agent with recognised vestibular effects, which the Unit regards as a plausible alternative explanation in those cases.

Laboratory data were available for nineteen of the reported patients. Serum potassium was within the reference range in seventeen of the nineteen at the time closest to symptom onset, and marginally elevated in two. An electrocardiogram had been recorded in six patients and showed no consistent abnormality. These figures are presented to characterise the reports; they describe what happened to be on file and are not a recommendation that either investigation be performed.

3. Conclusion

The Unit's conclusion is that the reports are consistent with a recognised, usually self-limiting effect of therapy initiation. **No change to laboratory monitoring practice arises from this advisory, and no new monitoring interval is introduced by it.** Existing requirements published elsewhere are unaffected and continue to apply on their own terms.

4. What clinicians should do

- Counsel patients starting CardioZen that transient dizziness is recognised and usually settles.
- Advise patients to report symptoms that persist beyond two weeks or that affect their ability to stand or walk safely.
- Continue to report suspected reactions through the usual channel.

5. Further work

The Unit will keep the signal under passive surveillance and will re-assess if the reporting rate changes materially. No routine data collection is being established for this purpose.

- End of advisory -